

Pembahasan Soal MPI-2

Berdasarkan Aturan Koding ICD-10

Disampaikan pada

Pelatihan Pengodean Diagnosis Penyakit dan Tindakan Bagi
Tenaga Koder di FPKTL dalam Program JKN

Bekasi, 10-15 November 2025



JAWABAN SOAL 1

	Nama Diagnosis	Kode ICD-10
Diagnosis Utama	Hipertensi dengan <i>Acute Decompensated Heart Failure (ADHF)</i>	I13.2
Diagnosis Sekunder	1. Syok Kardiogenik 2. Selulitis pedis dextra dd/ALI+ 3. Venous Thromboembolism (VTE) Femoralis pedis dextra 4. Chronic Kidney Disease (CKD) Stage V 5. Ventricular Tachycardia (VT) 6. Edema anasarka 7. Trombositopenia	- R57.0 - L03.1 - I80.1 - N18.5 - I47.2 - - D69.6





Diagnosis Utama :

Hipertensi dengan *Acute Decompensated Heart Failure (ADHF)*, disertai *Chronic Kidney Disease (CKD) Stage V*

Lead Term : Hypertension

- *Modifier :*

- *cardiorenal, with renal failure, and heart failure I13.2*

Hypertension, hypertensive
(accelerated) (benign) (essential)
(idiopathic) (malignant) (primary)
(systemic) I10

- **cardiorenal (disease) I13.9**

- - with

- - - renal failure I13.1

- - - - and heart failure (congestive) I13.2

I13 Hypertensive heart and renal disease

Includes: any condition in I11.- with any condition in I12.- disease:
· cardiorenal
· cardiovascular renal

I13.0 Hypertensive heart and renal disease with (congestive) heart failure

I13.1 Hypertensive heart and renal disease with renal failure

I13.2 Hypertensive heart and renal disease with both (congestive) heart failure and renal failure

I13.9 Hypertensive heart and renal disease, unspecified





Chronic Kidney Disease stage V **N18.5**

DRAFT ICS

ICS

INDONESIAN CODING STANDARD

**PEDOMAN
PENGODEAN IDRG**

VERSI 25072025

- Apabila pasien didiagnosis dengan penyakit ginjal hipertensi, ataupun penyakit jantung dan ginjal hipertensi, di mana penyakit ginjal termasuk dalam kategori N18.- Penyakit ginjal kronis, maka kode N18.- wajib dikode untuk mengidentifikasi tahap perkembangan penyakit ginjal kronis tersebut. Kode N18.- ini dapat digunakan bersamaan dengan kategori I12.- atau I13.-

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sesuai dengan kondisi pasien. Penentuan Urutan mana yang dijadikan diagnosa Utama dan sekunder mengacu pada chapter 2 mengenai definisi dan kriteria diagnosa utama & sekunder.





SYOK KARDIOGENIK **R57.0**

Shock R57.9

- adrenal (cortical) (Addisonian) E27.2
- adverse food reaction (anaphylactic) T78.0

- **cardiogenic R57.0**

R57.0 **Cardiogenic shock**

R57.1 **Hypovolaemic shock**

R57.2 **Septic shock**

R57.8 **Other shock**

Berita Acara Kesepakatan Bersama Panduan Penatalaksanaan Solusi Permasalahan Klaim INA-CBG Tahun 2019

Nomor : JP.02.03/3/1693/2020

Nomor : 411/BA/0720

B. ASPEK MEDIS

No	Diagnosis	Prosedur	Aspek Medis	Perhatian Khusus	Keterangan
1	Diagnosis sekunder Shock Kardiogenik pada kasus meninggal		Kondisi Syok Kardiogenik dapat menjadi diagnosis sekunder terutama pada pasien penyakit jantung dengan bukti tertulisnya kriteria klinis dalam rekam medis berupa : 1. Penurunan Tekanan Darah a. TD < 90 mmHg tanpa inotropik, atau b. TD < 80 mmHg dengan inotropik 2. Penurunan Ejection Fraction (EF < 50%)	Tidak boleh dikoding bila tidak tegak secara medis dan tidak ada resource khusus	BA Kesepakatan Bersama Tahun 2018





SELULITIS PEDIS DEXTRA **L03.1**

Cellulitis-----continued

- clostridial [A48.0](#)
- corpus cavernosum [N48.2](#)
- drainage site (following operation) [T81.4](#)
- ear (external) [H60.1](#)
- eosinophilic (granulomatous) [L98.3](#)
- erysipelatous (see also *Erysipelas*) [A46](#)
- external auditory canal [H60.1](#)
- eyelid [H00.0](#)
- face (any part, except ear, eye and nose) [L03.2](#)
- finger (intrathecal) (periosteal) (subcutaneous) (subcuticular) [L03.0](#)
- foot, except toe(s) [L03.1](#)

L03 Cellulitis

Includes: acute lymphangitis

Excludes: cellulitis of:

- anal and rectal regions ([K61.-](#))
- external auditory canal ([H60.1](#))
- external genital organs:
- female ([N76.4](#))
- male ([N48.2](#), [N49.-](#))
- eyelid ([H00.0](#))
- lacrimal apparatus ([H04.3](#))
- mouth ([K12.2](#))
- nose ([J34.0](#))
- eosinophilic cellulitis [Wells] ([L98.3](#))
- febrile neutrophilic dermatosis [Sweet] ([L98.2](#))
- lymphangitis (chronic)(subacute) ([I89.1](#))

L03.1

Cellulitis of other parts of limb

Axilla

Hip

Shoulder





VENOUS THROMBOEMBOLISM (VTE)

FEMORALIS PEDIS DEXTRA **I80.1**

Kesan / Kesimpulan USG Doppler :

**Gambaran sesuai dengan Trombosis
pada vena [femoralis] kanan
Vena Femoralis = Vena Dalam**

Thromboembolism (see also
Embolism) I74.9

Embolism (septic) I74.9

- vein I82.9
- - mesenteric (with gangrene) K55.0
- - portal I81
- - pulmonary — see *Embolism*,
pulmonary
- - renal I82.3
- - specified NEC I82.8

Referensi :

<https://my.clevelandclinic.org/health/body/23041-femoral-vein>

<https://www.globalradiologycme.com/single-post/2015/12/10/the-superficial-femoral-vein-is-a-deep-vein-hub>

I82

Other venous embolism and thrombosis

- Excludes:*
- venous embolism and thrombosis (of):
 - cerebral (I63.6 , I67.6)
 - complicating:
 - abortion or ectopic or molar pregnancy (O00-O07 , O08.7)
 - pregnancy, childbirth and the puerperium (O22.- , O87.-)
 - coronary (I21-I25)
 - intracranial and intraspinal, septic or NOS (G08)
 - intracranial, nonpyogenic (I67.6)
 - intraspinal, nonpyogenic (G95.1)
 - **lower extremities (I80.-)**
 - mesenteric (K55.0)
 - portal (I81)
 - pulmonary (I26.-)

I80.1

Phlebitis and thrombophlebitis of femoral vein





VENTRICULAR TACHYCARDIA **I47.2**

Tachycardia -----continued

- - paroxysmal [I47.1](#)
- sinus [I47.1](#)
- supraventricular [I47.1](#)
- ventricular [I47.2](#)

Tachypnea [R06.8](#)



I47.2 Ventricular tachycardia

TROMBOSITOPENIA **D69.6**

Thrombocytopenia, thrombocytopenic

D69.6

- with absent radius (TAR) [Q87.2](#)
- congenital [D69.4](#)
- dilutional [D69.5](#)



D69.4 Other primary thrombocytopenia

Excludes: thrombocytopenia with absent radius ([Q87.2](#))
transient neonatal thrombocytopenia ([P61.0](#))
Wiskott-Aldrich syndrome ([D82.0](#))

D69.5 Secondary thrombocytopenia

Use additional external cause code (Chapter XX), if desired, to identify cause.

D69.6 Thrombocytopenia, unspecified





Kenapa **Edema Anasarka (R60.1)** tidak dikodekan?

Anasarca R60.1

- cardiac (see also *Failure, heart, congestive*) I50.0
- fetus or newborn P83.2

see also petunjuk yang harus diikuti untuk mencegah kesalahan

Dalam dokumentasi disebutkan juga adanya Diagnosis Gagal Jantung (*Heart Failure*), sehingga dalam hal ini diagnosis Edema Anasarka tidak perlu di kodekan kembali.





JAWABAN SOAL 2

	Nama Diagnosis	Kode ICD-10
Diagnosis Utama	Adenoca Prostat	C61
Diagnosis Sekunder	1. Gagal Napas akut 2. Pneumonia 3. Sepsis karena Eschericia Coli 4. Chronic Kidney Disease Stage V karena Hipertensi tidak terkontrol 1. Anemia Keganasan	J96.0 J18.9 A41.5 I12.0 N18.5 D63.0*





ADENOCA PROSTAT C61

Adenocarcinoma (M8140/3) — see
also *Neoplasm, malignant*

Neoplasm, neoplastic – *continued*

- pituitary (body) (fossa) (gland) (lobe)...
- placenta.....
- pleura, pleural (cavity).....
- - parietal.....
- - visceral.....
- primary site unknown, so stated
- prostate (gland).....

Malignant		In situ	Benign	Uncertain or unknown behaviour
Primary	Secondary			
C75.1	C79.8	D09.3	D35.2	D44.3
C58	C79.8	D07.3	D26.7	D39.2
C38.4	C78.2	-	D15.7	D38.2
C38.4	C78.2	-	D15.7	D38.2
C38.4	C78.2	-	D15.7	D38.2
C80.0	C79.9			
C61	C79.8	D07.5	D29.1	D40.0

C61

Malignant neoplasm of prostate





GAGAL NAPAS AKUT J96.0

Failure, failed-----*continued*

- myocardial, myocardium (see also *Failure, heart*) I50.9
- organ — see Failure, by site
- - multiple NEC R68.8
- orgasm (female) (male) (psychogenic) F52.3
- ovarian (primary) E28.3
- - iatrogenic E89.4
- - postprocedural (postablative) (postirradiation) (postsurgical) E89.4
- ovulation causing infertility N97.0
- polyglandular, autoimmune E31.0
- renal —see *Failure, kidney*
- respiration, respiratory J96.9
- - - other organ failure, code each site (see also Failure organ multiple NEC)
- - acute J96.0

J96.0 Acute respiratory failure

PNEUMONIA J18.9



Pneumonia (acute) (double) (migratory) (purulent) (septic) (unresolved) J18.9

J18.9 Pneumonia, unspecified

Diskusi :

Ditemukan Kultur Sputum Bakteri *Acinetobacter baumannii* (Bakteri Aerob Gram Negatif)

Apakah dapat ditegakkan kode J15.6 ?
Pneumonia due to other aerobic Gram-negative bacteria





Dalam kasus ini Pengode (*coder*) memberikan kode sebagai berikut namun kurang tepat:

diagnosis primer : J15.0 *Pneumonia due to Klebsiella pneumoniae*

diagnosis sekunder : E10.9 *Insulin-dependent diabetes mellitus, without complication*

Z21 *Asymptomatic human immunodeficiency virus [HIV] infection status*

Semua kode dalam contoh di atas dianggap tidak tepat karena dinilai berlebihan (*over-coding*) dengan alasan berikut:

- Hasil kultur sputum tidak dapat dijadikan dasar pemberian kode, pengode (*coder*) harus konfirmasi kepada dokter mengenai kepastian diagnosa pneumonia dan dokter perlu melengkapinya dalam rekam medis mengenai diagnosa klebsiella pneumoniae.
 - Dalam kasus ini, kode J18.1 (*Lobar pneumonia*) seharusnya didasarkan pada laporan X-ray oleh dokter radiologi, bukan hasil kultur sputum.
 - Meskipun kultur sputum menemukan *Klebsiella pneumoniae*, hal ini hanya menunjukkan penyebab infeksi, bukan diagnosis klinis pneumonia itu sendiri.
 - Bila secara klinis dan pemeriksaan penunjang sudah ditegaskan oleh dokter dan terdokumentasi dalam rekam medis maka kode J15,0 dapat digunakan.

Pembahasan :

Ditemukan Kultur Sputum Bakteri *Acinetobacter baumannii* (Bakteri Aerob Gram Negatif) Namun tidak ditegakkan dalam Diagnosis Akhir, penulisan diagnosis akhir hanya Pneumonia sehingga kode yang tepat adalah J18.9, belum dapat ditegakkan kode J15.6

J15 Bacterial pneumonia, not elsewhere classified

Includes: bronchopneumonia due to bacteria other than *S. pneumoniae* and *H. influenzae*

Excludes: chlamydial pneumonia ([J16.0](#))
congenital pneumonia ([P23.-](#))
Legionnaires' disease ([A48.1](#))

J15.0 Pneumonia due to Klebsiella pneumoniae

J15.1 Pneumonia due to Pseudomonas

J15.2 Pneumonia due to staphylococcus

J15.3 Pneumonia due to streptococcus, group B

J15.4 Pneumonia due to other streptococci

Excludes: pneumonia due to:
· streptococcus, group B ([J15.3](#))
· *Streptococcus pneumoniae* ([J13](#))

J15.5 Pneumonia due to Escherichia coli

J15.6 Pneumonia due to other aerobic Gram-negative bacteria Pneumonia due to *Serratia marcescens*

J15.7 Pneumonia due to Mycoplasma pneumoniae

J15.8 Other bacterial pneumonia

J15.9 Bacterial pneumonia, unspecified





Disease, diseased-----continued

- kidney (functional) (pelvis) N28.9

Note: Where a term is indexed only at the three-character level, e.g. N01.-, reference should be made to the list of fourth-character subdivisions in Volume 1 at N00-N08.

- - hypertensive (see also Hypertension, kidney) I12.9
- - - end-stage (failure) I12.0

Hypertension, hypertensive

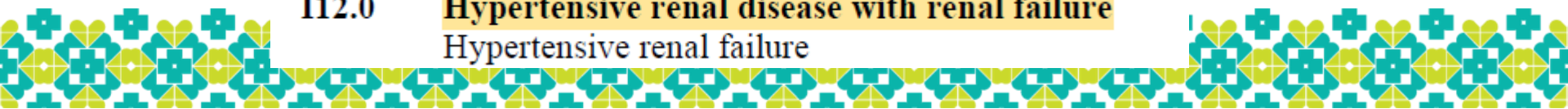
(accelerated) (benign) (essential) (idiopathic) (malignant) (primary) (systemic) I10

- kidney I12.9
- - with
- - - heart involvement (conditions in I51.4-I51.9 due to Hypertension) (see also *Hypertension, cardiorenal*) I13.9
- - - - with heart failure (congestive) I13.0
- - - - - with renal failure I13.2
- - - hypertensive heart disease (conditions in I11.-) (see also *Hypertension, cardiorenal*) I13.9
- - - - with heart failure (congestive) I13.0
- - - - - with renal failure I13.2
- - - renal failure I12.0

I12.0

Hypertensive renal disease with renal failure

Hypertensive renal failure





Chronic Kidney Disease stage V **N18.5**

DRAFT ICS

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PENGODEAN IDRG**

VERSI 25072025

- Apabila pasien didiagnosis dengan penyakit ginjal hipertensi, ataupun penyakit jantung dan ginjal hipertensi, di mana penyakit ginjal termasuk dalam kategori N18.- Penyakit ginjal kronis, maka kode N18.- wajib dikode untuk mengidentifikasi tahap perkembangan penyakit ginjal kronis tersebut. Kode N18.- ini dapat digunakan bersamaan dengan kategori I12.- atau I13.-

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sesuai dengan kondisi pasien. Penentuan Urutan mana yang dijadikan diagnosa Utama dan sekunder mengacu pada chapter 2 mengenai definisi dan kriteria diagnosa utama & sekunder.





SEPSIS KARENA BAKTERI ESCHERICIA COLI **A41.5**

Escherichia coli (E. coli) adalah bakteri gram negatif

Referensi : <https://www.ncbi.nlm.nih.gov/books/NBK564298/>

Sepsis-----continued

- anaerobic **A41.4**
- Bacillus anthracis **A22.7**
- bacterial, newborn **P36.9**
- during labour **O75.3**
- Erysipelothrix (erysipeloid) (rhusiopathiae) **A26.7**
- Escherichia coli **A41.5**

A41.5 Sepsis due to other Gram-negative organisms

Gram-negative sepsis NOS

BA 2018 Aspek Medis No.43 - Septikemia

Penegakan diagnosis sepsis dapat mengikuti kriteria SIRS (systemic inflammatory response syndrome) yaitu terdiri dari minimal 2 keadaan:

1. Temperatur >38,5 derajat celcius atau <36 derajat celcius
2. Denyut Jantung >90 x/menit
3. Frekuensi pernafasan >20x/menit atau PaCO₂ <32 mmHg (Pada pemeriksaan AGDA)
4. Terdapat respons tubuh terhadap fokal infeksi, peradangan, dan stres dengan hasil laboratorium menunjukkan leukositosis dan wajib melampirkan bukti kultur darah dengan hasil bakterimia.

Apabila diagnosis sepsis dapat ditegakkan maka harus diikuti dengan tata laksana sepsis.





ANEMIA KEGANASAN **D63.0**

Anemia-----*continued*

- hypochromic **D50.9**
- - familial sex-linked **D64.0**
- - microcytic **D50.8**
- - normoblastic **D50.8**
- - pyridoxine-responsive **D64.3**
- - sideroblastic, sex-linked **D64.0**
- hypoplasia, red blood cells **D61.9**
- - congenital or familial **D61.0**
- hypoplastic **D61.9**
- - congenital or familial **D61.0**

- in

- - chronic kidney disease
- - - stage 3 **N18.3† D63.8***
- - - stage 4 **N18.4† D63.8***
- - - stage 5 **N18.5† D63.8***
- - - unspecified **N18.9† D63.8***
- - neoplastic disease NEC (**M8000/1**)
(see also *Neoplasm*) **D48.9† D63.0***

D63***Anaemia in chronic diseases classified elsewhere****D63.0*****Anaemia in neoplastic disease (**C00-D48†**)**



JAWABAN SOAL 3

	Nama Diagnosis	Kode ICD-10
Diagnosis Utama	G2P1A0 36 Minggu 5 hari T/H dengan Pre Eklamsia Berat (PEB)	O14.1
Diagnosis Sekunder	1. Ketuban Pecah Dini (KPD) 3 jam 2. DM Tipe 2 dengan Obesitas grade 2 3. Persalinan Prematur 4. <i>Post Seksio Cesarean Emergensi ..</i> 5. <i>Bayi Lahir Tunggal Hidup..</i>	- O42.0 - O24.2 - O60.3 - O82.1 - Z37.0



Diagnosis Utama : Pre Eklamsia Berat (PEB)

Lead Term : Pre-eclampsia

Pre-eclampsia O14.9

- **Modifier** : Severe (Berat)

- with pre-existing hypertension O11
- affecting fetus or newborn P00.0
- mild O13
- moderate O14.0
- severe O14.1
- superimposed O11

O14.1 Severe pre-eclampsia

Diagnosis Sekunder 1:

Ketuban Pecah Dini (KPD) 3 jam
(Premature Rupture Of Membrane (PROM))

Lead Term : Rupture

- **Modifier** :

Membranes, Premature, onset of labor, within 24 hours

O42 Premature rupture of membranes

O42.0 Premature rupture of membranes, onset of labour within 24 hours

SOAL 3

Rupture, ruptured-----continued

- intestine NEC (nontraumatic) K63.1

- membranes (spontaneous)

- - artificial

- - - delayed delivery following O75.5

- - - - affecting fetus or newborn P01.1

- - delayed delivery following O75.6

- - - affecting fetus or newborn P01.1

- - premature O42.9

- - - affecting fetus or newborn P01.1

- - - labor delayed by therapy O42.2

- - - onset of labor

- - - - after 24 hours O42.1

- - - - delayed by therapy O42.2

- - - - within 24 hours O42.0



Diagnosis Sekunder 2 :

Diabetes Mellitus Tipe 2 dengan Obesitas Grade 2 (in Pregnancy)

Lead Term : Diabetes, diabetic

- **Modifier :**

- complicating pregnancy, childbirth or puerperium (maternal) O24.-
- - pre-existing O24.3
- - - malnutrition-related O24.2

Ringkasan

Malnutrisi mengacu pada kekurangan, kelebihan, atau ketidakseimbangan asupan energi dan/atau zat gizi seseorang. Istilah malnutrisi mencakup 3 kelompok kondisi umum:

- kekurangan gizi, yang mencakup wasting (berat badan rendah dibandingkan tinggi badan), stunting (tinggi badan rendah dibandingkan usia) dan underweight (berat badan rendah dibandingkan usia);
- malnutrisi terkait mikronutrien, yang mencakup kekurangan mikronutrien (kekurangan vitamin dan mineral penting) atau kelebihan mikronutrien; dan
- kelebihan berat badan, obesitas, dan penyakit tidak menular yang terkait dengan pola makan (seperti penyakit jantung, stroke, diabetes, dan beberapa jenis kanker).

Malnutrition References :

<https://www.who.int/news-room/fact-sheets/detail/malnutrition>
<https://pmc.ncbi.nlm.nih.gov/articles/PMC8820192/>

Diabetes, diabetic (mellitus) (controlled) (familial) (severe) E14.-

- acetoneuria — code to E10-E14 with fourth character . 1

- complicating pregnancy, childbirth or puerperium (maternal) O24.-

- - affecting fetus or newborn P70.1
- - arising in pregnancy O24.4
- - - affecting fetus or newborn P70.0
- - gestational O24.4
- - - affecting fetus or newborn P70.0

- - pre-existing O24.3

- - - insulin-dependent O24.0

- - - malnutrition-related O24.2

- - - non-insulin-dependent O24.1

O24.2 Pre-existing malnutrition-related diabetes mellitus





Diagnosis Sekunder 3 : Persalinan Prematur

Lead Term : Premature

- **Modifier :**

- - without spontaneous labor (cesarean section (induction) O60.3 -

Premature - see also condition

- adrenarache E27.0
- aging E34.8
- beats I49.4
- - atrial I49.1
- birth NEC P07.3
- closure, foramen ovale Q21.8
- contraction
- - atrial I49.1
- - atrioventricular I49.2
- - heart (extrasystole) I49.4
- - junctional I49.2
- - ventricular I49.3
- delivery (with spontaneous labor) NEC O60.1
- - without spontaneous labor (cesarean section) (induction) O60.3

Kode Sekunder 4 : Persalinan Sectio Cesarean

Lead Term : Cesarean

- **Modifier :** *Emergency*

- Cesarean**
- emergency O82.1
- operation or section NEC (see also *Delivery, cesarean*) O82.9
- - with hysterectomy O82.2
- - affecting fetus or newborn P03.4

Kode Sekunder 5 : Lahir Tunggal Hidup

Lead Term : Outcome of Delivery

- **Modifier :** *single liveborn*

- Outcome of delivery** Z37.9
- multiple (births) Z37.9
- - all liveborn Z37.5
- - all stillborn Z37.7
- - some liveborn Z37.6
- single Z37.9
- - liveborn Z37.0

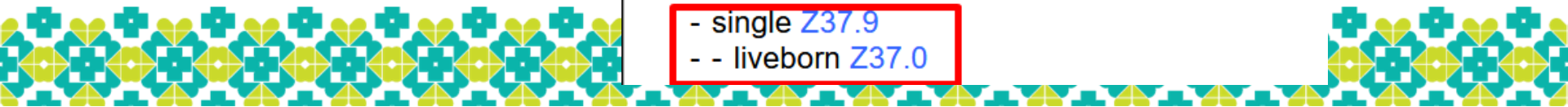
SOAL 3

Catatan Rekam Medis :

Pasien datang dari IGD

diantar oleh keluarga dengan keluhan Nyeri perut hilang timbul (-) dari DPJP dilaksanakan tindakan SC a.i PEB

SC terlaksana dengan baik, bayi lahir utuh laki-laki BBL : 2900 gr, menangis kencang





O60.3

Preterm delivery without spontaneous labour ⚡

Preterm delivery by:

- caesarean section, without spontaneous labour
- induction

O82.1

Delivery by emergency caesarean section

Z37

Outcome of delivery

Note: This category is intended for use as an additional code to identify the outcome of delivery on the mother's record.

Z37.0

Single live birth



3.14.23 Delivery (O80-O84)

Untuk memilih kode diagnosis utama bagi pasien yang melahirkan, pertimbangkan urutan prioritas berikut:

- Urutan Pertama: Jika persalinan dilakukan dengan tindakan obstetrik, seperti operasi caesar, penggunaan forsep, penggunaan alat vakum, atau bantuan persalinan sungsang, gunakan kode indikasi untuk tindakan obstetrik tersebut sebagai kode diagnosis utama.
- Urutan Kedua: Jika persalinan normal tetapi ada penyakit penyerta atau komplikasi selama perawatan, gunakan kode untuk diagnosis penyerta atau komplikasi yang paling parah menurut dokter, yang terdapat dalam kategori huruf "O", sebagai kode diagnosis utama.
- Urutan Ketiga: Jika persalinan normal tanpa penyakit penyerta atau komplikasi, gunakan kode metode persalinan (O80 - O84) sebagai kode diagnosis utama.

Dengan demikian, kode metode persalinan (O80 - O84) hanya dapat digunakan sebagai kode diagnosis utama jika tidak ada kode diagnosis lain yang dimulai dengan huruf "O" pada episode perawatan ini. Artinya, hanya berlaku untuk kasus persalinan normal tanpa penyakit penyerta atau komplikasi. Berdasarkan prinsip ini, diagnosis utama, diagnosis sekunder, atau komplikasi yang dirangkum oleh dokter di rekam medis mungkin berbeda dari kode yang diberikan oleh pengode. Baik dokter maupun pengode tidak salah dalam hal ini.

Gunakan kode hasil persalinan dalam kelompok Z37.- *Outcome of delivery* sebagai kode diagnosis sekunder untuk semua pasien yang melahirkan. Kode dalam kelompok ini tidak boleh digunakan sebagai kode diagnosis utama dalam situasi apa pun. Bahkan jika dokter tidak mencatat hasil persalinan dalam resume medis (ringkasan pulang), adalah tanggung jawab pengode untuk mencari informasi hasil persalinan dari bagian lain rekam medis, seperti catatan persalinan atau ringkasan persalinan.

j. Pengodean Persalinan (O80-O84).

- 1) Kode-kode O80-O84 (*Delivery*) digunakan sebagai diagnosis sekunder untuk menunjukkan metode persalinan.
- 2) Jika tidak terdapat komplikasi atau penyulit persalinan lainnya maka kode O80-O84 (*Delivery*) digunakan sebagai diagnosis utama.
- 3) Kode Z37.0 – Z37.9 (*Outcome of delivery*) digunakan sebagai diagnosis sekunder.

Referensi Berkaitan dengan Kaidah Koding Klaim pada Kasus Persalinan



JAWABAN SOAL 4

	Nama Diagnosis	Kode ICD-10
Diagnosis Utama	Penyakit Jantung Hipertensi dengan Gagal Ginjal on HD	I13.2 N18.5
Diagnosis Sekunder	1. Edema Paru Akut 2. Anemia renal 3. Infeksi post pemasangan CDL 4. DM tipe II 5. Gagal Jantung	- D63.8* T82.7 , Y83.1 E11.9 -





Diagnosis Utama :

Penyakit Jantung Hipertensi dengan Gagal Ginjal on HD disertai Gagal Jantung

Lead Term : Hypertension

- **Modifier :**

- *cardiorenal, with renal failure, and heart failure I13.2*

Hypertension, hypertensive
(accelerated) (benign) (essential)
(idiopathic) (malignant) (primary)
(systemic) **I10**

- **cardiorenal (disease) I13.9**

- - with

- - - renal failure **I13.1**

- - - - and heart failure (congestive) **I13.2**

I13 Hypertensive heart and renal disease

Includes: any condition in I11.- with any condition in **I12.-** disease:
· cardiorenal
· cardiovascular renal

I13.0 Hypertensive heart and renal disease with (congestive) heart failure

I13.1 Hypertensive heart and renal disease with renal failure

I13.2 Hypertensive heart and renal disease with both (congestive) heart failure and renal failure

I13.9 Hypertensive heart and renal disease, unspecified





Chronic Kidney Disease stage V **N18.5**

DRAFT ICS

ICS

INDONESIAN CODING STANDARD

**PEDOMAN
PENGODEAN IDRG**

VERSI 25072025

- Apabila pasien didiagnosis dengan penyakit ginjal hipertensi, ataupun penyakit jantung dan ginjal hipertensi, di mana penyakit ginjal termasuk dalam kategori N18.- Penyakit ginjal kronis, maka kode N18.- wajib dikode untuk mengidentifikasi tahap perkembangan penyakit ginjal kronis tersebut. Kode N18.- ini dapat digunakan bersamaan dengan kategori I12.- atau I13.-

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sesuai dengan kondisi pasien. Penentuan Urutan mana yang dijadikan diagnosa Utama dan sekunder mengacu pada chapter 2 mengenai definisi dan kriteria diagnosa utama & sekunder.





Kenapa **Edema Paru Akut** tidak dikodekan?

DRAFT ICS.V1

Contoh 1:

DU : Hipertensi dengan gagal ginjal yang disertai gagal jantung

DS : edema paru

Contoh : Hipertensi dengan gagal ginjal yang disertai dengan gagal jantung dengan diagnosis sekunder edema paru, maka menggunakan kode gabungan I13.2 *Hypertensive heart and renal disease with both (congestive) heart failure and renal failure* dan edema paru tidak perlu dikoding terpisah.

BA 2018 ; Aspek Koding No. 61

Sesuai kaidah koding, Hipertensi dengan gagal ginjal yang disertai gagal jantung, maka udem paru tidak dikoding terpisah dan dikode I13.2.
Jika diagnosis utama hipertensi dengan gagal ginjal maka dikode I12.0 dan Udem paru (J81) dikode tersendiri. Walaupun secara klinis, udem paru merupakan bagian dari tanda dan gejala dari acute on Chronic renal failure (overload syndrome)

Kriteria Pulmonary Oedema: gejala klinis sesak, takikardi, ronki
Ada penatalaksanaan pulmonary oedema yang terekam dalam resume medis dan ada terapi diuretik dan oksigen yang diberikan.

BA Kesepakatan Bersama Tahun 2018





Anemia Renal (D63.8*)

Anemia D64.9

- with disorder of
- - anaerobic glycolysis D55.2
- - pentose phosphate pathway D55.1
- achlorhydric D50.8
- achrestic D53.1

- - hemolytic, chronic D59.9

- in

- - chronic kidney disease

- - - stage 3 N18.3† D63.8*

- - - stage 4 N18.4† D63.8*

- - - stage 5 N18.5† D63.8*

- - - unspecified N18.9† D63.8*

D63*

Anaemia in chronic diseases classified elsewhere

D63.0*

Anaemia in neoplastic disease ([C00-D48†](#))

D63.8*

Anaemia in other chronic diseases classified elsewhere

Anaemia in chronic kidney disease $\geq$ stage 3 ([N18.3 – N18.5†](#))





Infeksi post pemasangan CDL (T82.7, Y83.1)

Infection, infected (opportunistic) B99

Note: Parasitic diseases may be described as either 'infections' or 'infestations'; both lead terms should therefore be consulted.

- due to or resulting from
 - - artificial insemination N98.0
 - - device, implant or graft (*see also Complications, by site and type*) T85.7
 - - - arterial graft NEC T82.7
 - - - breast (implant) T85.7
 - - - catheter NEC T85.7
 - - - dialysis (renal) T82.7

T82.7 Infection and inflammatory reaction due to other cardiac and vascular devices, implants and grafts





Infeksi post pemasangan CDL (T82.7, Y83.1)

Complication (delayed) (of or following) (medical or surgical procedure) [Y84.9](#)

- implant, implantation (of)
- - artificial
- - - internal device (cardiac pacemaker) (electrodes in brain) (heart valve prosthesis) (orthopedic) [Y83.1](#)

Surgical and other medical procedures as the cause of abnormal reaction of the patient, or of later complication, without mention of misadventure at the time of the procedure (Y83-Y84)

Excludes: misadventures to patients during surgical and medical care, classifiable to Y60-Y69 ([Y60-69](#))
breakdown or malfunctioning of medical device (during procedure) (after implantation) (ongoing use) ([Y70-Y82](#))

Section II External Causes Of Injury

► **Section I Alphabetical Index Of Diseases And Nature Of Injury**

► **Section II External Causes Of Injury**

► **Section III Table Of Drugs And Chemicals**

Y83 **Surgical operation and other surgical procedures as the cause of abnormal reaction of the patient, or of later complication, without mention of misadventure at the time of the procedure**

Y83.0 **Surgical operation with transplant of whole organ**

Y83.1 **Surgical operation with implant of [artificial internal device](#)**





Kenapa **Y83.1** perlu dikodekan?

DRAFT ICS.V1 Hal. 479

3.23.8.25 Surgical And Other Medical Procedures As The Cause Of Abnormal Reaction Of The Patient, Or Of Later Complication, Without Mention Of Misadventure At The Time Of The Procedure (Y83-Y84)

Terapkan kode dari kelompok Y83 – Y84 sebagai penanda penyebab eksternal dalam rekam medis, dengan memastikan bahwa pemilihan kode mencerminkan secara tepat konteks dan detail kejadian yang terjadi. Kode ini dirancang untuk menggambarkan faktor-faktor eksternal yang secara langsung atau tidak langsung berkontribusi pada kondisi medis pasien, seperti komplikasi atau insiden selama perawatan. Oleh karena itu, dokumentasi harus disusun secara kritis dan rinci, mencakup informasi penting seperti jenis tindakan medis, waktu kejadian, dan situasi spesifik yang menyertainya.

ICD-10-Revisi Tahun 2010

Complications of surgical and medical care, not elsewhere classified (T80-T88)

Use additional external cause code (Chapter XX), if desired, to identify devices involved and details of circumstances.

Use additional code ([B95-B97](#)), if desired, to identify infectious agent.





Diabetes Mellitus Tipe 2 (E11.9)

Diabetes, diabetic-----*continued*

- non-insulin-dependent (of the young) E11.-
- nonketotic E11.-

E11 Non-insulin-dependent diabetes mellitus

[\[See before E10 for subdivisions\]](#)

Includes: diabetes (mellitus)(nonobese)(obese):

- adult-onset
- maturity-onset
- nonketotic
- stable
- type II

non-insulin-dependent diabetes of the young

Diabetes mellitus (E10-E14)

Use additional external cause code (Chapter XX), if desired, to identify drug, if drug-induced.

The following fourth-character subdivisions are for use with categories E10-E14:

.9 Without complications





JAWABAN SOAL 5

	Nama Diagnosis	Kode ICD-10
Diagnosis Utama	Infark Cerebri karena Multiple Stenosis Arteri Cerebri	I63.5
Diagnosis Sekunder	1. Oklusi total pada LtMCA segmen M1 sampai distal 2. Hemiplegia dextra	- G81.9





Infark Cerebri karena Multiple Stenosis Arteri Cerebri (I63.5)

Infarct, infarction (of)

- adrenal (capsule) (gland) [E27.4](#)
- bowel [K55.0](#)
- brain (*see also* [Infarct, cerebral](#)) [I63.9](#)
- - embolic [I63.4](#)
- - puerperal, postpartum or in childbirth or pregnancy [O99.4](#)
- breast [N64.8](#)
- cardiac (*see also* [Infarct, myocardium](#)) [I21.9](#)
- cerebellar (*see also* [Infarct, cerebral](#)) [I63.9](#)
- - embolic [I63.4](#)
- - cerebral (hemorrhagic) [I63.9](#)
- - - due to
- - - - cerebral venous thrombosis, nonpyogenic [I63.6](#)
- - - - embolism (hemorrhagic)
- - - - - cerebral arteries [I63.4](#)
- - - - - precerebral arteries [I63.1](#)
- - - - occlusion NEC
- - - - - cerebral arteries [I63.5](#)
- - - - - precerebral arteries [I63.2](#)
- - - stenosis NEC
- - - - cerebral arteries [I63.5](#)
- - - - - precerebral arteries [I63.2](#)
- - - thrombosis (hemorrhagic)
- - - - cerebral arteries [I63.3](#)
- - - - - precerebral arteries [I63.0](#)

I63 Cerebral infarction

Includes: occlusion and stenosis of cerebral and precerebral arteries, resulting in cerebral infarction

Excludes: sequelae of cerebral infarction ([I69.3](#))

- I63.0** Cerebral infarction due to thrombosis of precerebral arteries
- I63.1** Cerebral infarction due to embolism of precerebral arteries
- I63.2** Cerebral infarction due to unspecified occlusion or stenosis of precerebral arteries
- I63.3** Cerebral infarction due to thrombosis of cerebral arteries
- I63.4** Cerebral infarction due to embolism of cerebral arteries
- I63.5** Cerebral infarction due to unspecified occlusion or stenosis of cerebral arteries
- I63.6** Cerebral infarction due to cerebral venous thrombosis, nonpyogenic
- I63.8** Other cerebral infarction
- I63.9** Cerebral infarction, unspecified



Ketika dokter mendiagnosis infark serebral, kode harus diberikan secara rinci dalam grup I63.- untuk menentukan apakah kondisi tersebut disebabkan oleh emboli atau trombosis, serta untuk menentukan lokasi pembuluh darah, apakah di arteri serebral atau precerebral.

Selain itu, kode tambahan dari grup **G46.-*** *Vascular syndrome of brain in cerebrovascular disease* harus diberikan sebagai diagnosis tambahan untuk menentukan pembuluh darah mana yang terkena kelainan tersebut.

G46* Vascular syndromes of brain in cerebrovascular diseases ([I60-I67†](#))

G46.0* Middle cerebral artery syndrome ([I66.0†](#))

G46.1* Anterior cerebral artery syndrome ([I66.1†](#))

G46.2* Posterior cerebral artery syndrome ([I66.2†](#))

Note :

Dalam kasus ini tidak didokumentasikan secara detail cerebral syndrome, meski diketahui level oklusinya namun **kode G46.-* belum dapat diterapkan karena tidak terdokumentasikan**

Syndrome-----continued

- - postsurgical [K91.2](#)
- blue diaper [E70.8](#)
- brachial plexus [G54.0](#)
- brain [F06.9](#)
- cerebral artery
- - anterior [I66.1†](#) **G46.1***
- - middle [I66.0†](#) **G46.0***
- - posterior [I66.2†](#) **G46.2***

Kenapa **Oklusi total pada LtMCA segmen M1** sampai distal tidak dikodekan?

Occlusion, occluded

- artery — see also *Embolism, artery*

- - cerebral **I66.9**

- - - with infarction (due to) **I63.5**

- - - - embolism **I63.4**

- - - - thrombosis **I63.3**

- - - anterior **I66.1**

- - - - with infarction (due to) **I63.5**

- - - - - embolism **I63.4**

- - - - - thrombosis **I63.3**

- - bilateral **I66.4**

- - - middle **I66.0**

- - - - with infarction (due to) **I63.5**

- - - - - embolism **I63.4**

- - - - - thrombosis **I63.3**

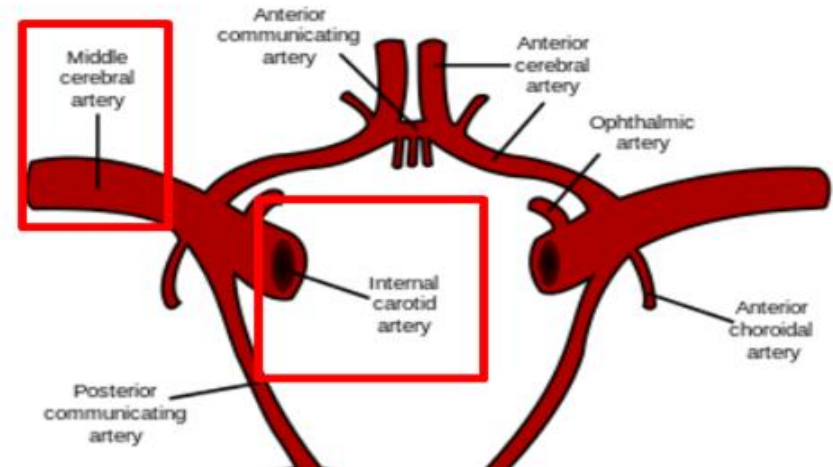
I66 Occlusion and stenosis of cerebral arteries, not resulting in cerebral infarction

Includes: embolism
narrowing
obstruction
(complete)(partial)
thrombosis

of middle, anterior and posterior cerebral arteries, and cerebellar arteries, not resulting in cerebral infarction

Excludes: when causing cerebral infarction (**I63.-**)

I66.0 Occlusion and stenosis of middle cerebral artery





Hemiparese dextra **G81.9**

Hemiparalysis (see also *Hemiplegia*)

G81.9

Hemiparesis (see also *Hemiplegia*)

G81.9

Hemiparesthesia **R20.2**

Hemiparkinsonism **G20**

Hemiplegia **G81.9**

- alternans facialis **G83.8**
- ascending NEC **G81.9**
- - spinal **G95.8**
- congenital (cerebral) **G80.8**
- - spastic **G80.2**
- embolic (current episode) **I63.4**
- flaccid **G81.0**
- hysterical **F44.4**
- newborn NEC **P91.8**
- - birth injury **P11.9**
- spastic **G81.1**
- - congenital **G80.2**
- spinal **G81.1**
- thrombotic (current episode) **I63.3**

G81 **Hemiplegia**

Note: For primary coding, this category is to be used only when hemiplegia (complete) (incomplete) is reported without further specification, or is stated to be old or longstanding but of unspecified cause. The category is also for use in multiple coding to identify these types of hemiplegia resulting from any cause.

Excludes: congenital cerebral palsy (**G80.-**)

G81.0 **Flaccid hemiplegia**

G81.1 **Spastic hemiplegia**

G81.9 **Hemiplegia, unspecified**

BA 2018 Aspek Medis No. 20

Tidak semua kasus stroke disertai dengan Hemiplegia/Hemiparese.

Untuk pasien dengan Hemiplegia di rawat inap ada tindakan fisioterapi.





JAWABAN SOAL 6

	Nama Diagnosis	Kode ICD-10
Diagnosis Utama	Obstruksi DJ Stent Kanan	T83.1
Diagnosis Sekunder	1. Batu Ginjal kiri 2. Batu Ureter kiri 1. Batu Buli 1. Infeksi Salurrah Kemih (ISK)	- N20.2 - NL12 (IM) (Ginjal Kiri) - NL14 (IM) (Ureter Kiri) - N21.0 - NL15 (IM) (Bladder) -





OBSTRUksi DJ STENT KANAN T83.1

Obstruction,... -----continued

- - rheumatic I06.0
- - aqueduct of Sylvius G91.1
- - congenital Q03.0
- - - with spina bifida (see also *Spina bifida, with hydrocephalus*) Q05.4
- - - valve (prosthesis) T82.0
- - - - graft T82.2
- - joint prosthesis T84.0
- - orthopedic NEC T84.4
- - specified NEC T85.6
- - **urinary NEC T83.1**
- - - graft T83.2



mechanical, narrowing

T83.1 Mechanical complication of other urinary devices and implants

Conditions listed in T82.0 due to:

- urinary:
 - electronic stimulator device
 - sphincter implant
 - stent

T82.0 Mechanical complication of heart valve prosthesis

Breakdown (mechanical)
Displacement
Leakage
Malposition
Obstruction, mechanical
Perforation
Protrusion

} due to heart valve prosthesis





BATU GINJAL KIRI dan BATU URETER KIRI N20.2

Calculus, calculi, calculous

- kidney (impacted) (multiple) (pelvis) (recurrent) (staghorn) [N20.0](#)
- - with calculus, ureter — see *Calculus, ureter, with calculus, kidney*
- renal (impacted) (recurrent) (see also *Calculus, kidney*) [N20.0](#)
- ureter (impacted) (recurrent) [N20.1](#)
- - with
- - - calculus, kidney [N20.2](#)



N20 Calculus of kidney and ureter

Includes: calculous pyelonephritis

Excludes: with hydronephrosis ([N13.2](#))

N20.0 Calculus of kidney

Nephrolithiasis NOS
Renal calculus or stone
Staghorn calculus
Stone in kidney

N20.1 Calculus of ureter

Ureteric stone

N20.2 Calculus of kidney with calculus of ureter

BATU BULI (BLADDER) N21.0

Calculus, calculi, calculous

- bladder (diverticulum) (encysted) (impacted) (urinary) [N21.0](#)



N21.0 Calculus in bladder

Calculus in diverticulum of bladder
Urinary bladder stone

Excludes: staghorn calculus ([N20.0](#))





LATERALISASI BATU UROLITHIASIS DALAM ICS

DRAFT ICS.V1

lateralization of urinary anatomy code:

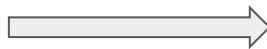
NL11 Right kidney (IM)

NL12 Left kidney (IM)

NL13 Right ureter (IM)

NL14 Left ureter (IM)

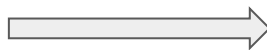
NL15 Bladder (IM)



BATU GINJAL KIRI



BATU URETER KIRI



BATU BULI / BLADDER





Kenapa **Infeksi Salurrah Kemih (ISK)** tidak dikodekan?

Other diseases of urinary system (N30-N39)

Excludes: urinary infection (complicating):

- abortion or ectopic or molar pregnancy ([O00-O07](#), [O08.8](#))
- pregnancy, childbirth and the puerperium ([O23.-](#), [O75.3](#), [O86.2](#))
- with urolithiasis ([N20-N23](#))





Kenapa Infeksi Salurrah Kemih (ISK) tidak dikodekan?

DRAFT ICS.V1

3.11.7 Urolithiasis N20-N23

Berikan kode dari kelompok N20.- *Calculus of kidney and ureter* dan N21.- *Calculus of lower urinary tract* sesuai dengan lokasi batu seperti yang didiagnosis oleh dokter.

- Batu pada ginjal, maka gunakan kode N20.0 *Calculus of kidney*.
- Batu saluran kemih dengan disertai ISK, maka gunakan kode gabungan pada sub bab *Urolithiasis* (N20-N23) sesuai dengan kriteria *excludes* yang dijelaskan pada sub bab N30-N39. (Hal ini sesuai pada BA tahun 2023)

BA 2018 ; Aspek Koding No. 108

N20.0 Calculus of kidney termasuk didalamnya: Nephrolithiasis NOS Renal calculus or stone Staghorn calculus Stone in kidney Perhatikan pada kategori N20 Includes : calculus pyelonephritis Excludes : with hydronephrosis (N13.2) -with infection (N13.6) Sesuai dengan instruksi excludes pada ICD 10 2008 dan 2010 volume I sub bab other diseases of urinary system (N30-N39) yang menyatakan urinary infection (complicating) with urolithiasis mengarah pada satu kode (N20-N23). Kondisi infeksi pada urinary calculus tidak menggunakan kode tersendiri. Cukup dikode urinary calculus sebagai diagnosa utama (tanpa diagnosa sekunder)	Kondisi batu saluran kemih dan batu ginjal yang disertai hidronefrosis dan infeksi menggunakan kode N13.6 Kondisi batu saluran kemih yang disertai dengan infeksi seluruh kemih menggunakan kode gabungan di N20-N23 (sesuai kriteria eksklusi di sub bab N30-N39)	BA Kesepakatan Bersama Tahun 2018
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BA 2023 ; Aspek Koding No. 03

3	Kondisi infeksi saluran kemih (N39.0) yang sudah include dalam kondisi Batu saluran kemih (N20-N23) namun ditagihkan terpisah. Seharusnya kondisi batu saluran kemih yang disertai dengan infeksi saluran kemih menggunakan kode gabungan di N20- N23 (sesuai kriteria eksklusi di sub bab N30- N39).	DU: N20-N30 DS: N39.0	DU: N20-N30 DS: -	kondisi batu saluran kemih yang disertai dengan infeksi saluran kemih menggunakan kode gabungan di N20-N23 (sesuai kriteria eksklusi di sub bab N30- N39).
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JAWABAN SOAL 7

	Nama Diagnosis	Kode ICD-10
Diagnosis Utama	Tuberkulosis Paru Gagal Terapi Xtreme-Drug-Resistance (XDR) dengan Pneumothorax (TCM +) HIV (-) (R,Lfx, Mfx, Bdq, Cfz) Resistance : (Rifampicin, Levofloxacin, Moxifloxacin , Bedaquiline, Clofazimine)	A15.12 U88
Diagnosis Sekunder	Bronkiektasis Use Additional Code : <i>Respiratory diseases with resistance to other antibiotics (IM)</i> <i>Tuberculosis with extreme drug resistance (IM))</i>	- U83.3 (IM) U84.33 (IM)





Tuberkulosis Paru (TCM +) [A15.12]

ICD 10 Revisi Tahun 2010

A15 Respiratory tuberculosis, bacteriologically and histologically confirmed

A15.0 Tuberculosis of lung, confirmed by sputum microscopy with or without culture

Tuberculous:

- bronchiectasis
- fibrosis of lung
- pneumonia
- pneumothorax

confirmed by sputum microscopy with or without culture

A15.1 Tuberculosis of lung, confirmed by culture only

Conditions listed in A15.0, confirmed by culture only

DRAFT ICD-10-IM

A15.1 Tuberculosis of lung, confirmed by culture only

Conditions listed in A15.0, confirmed by culture only

A15.10 Tuberculosis of lung, with previously treated confirmation culture only ^(IM)

A15.11 Tuberculosis of lung, confirmed by biomolecular examination ^(IM)

A15.12 Tuberculosis of lung, with previously treated confirmed by biomolecular examination ^(IM)

Tuberculosis, tubercular -----continued

- pneumothorax (spontaneous) (tense valvular) — see *Tuberculosis, pulmonary*
- pulmonalis — see *Tuberculosis, pulmonary*
- pulmonary (cavitated) (fibrotic) (infiltrative) (nodular) **A16.2**
- - bacteriological and histological examination not done **A16.1**
- - bacteriologically and histologically negative **A16.0**
- - childhood type or first infection **A16.7**
- - - with bacteriological and histological confirmation **A15.7**
- - confirmed (by)
- - - culture only **A15.1**





Kenapa TB Paru dikodekan A15.12?

DRAFT ICD-10-IM

A15.1 Tuberculosis of lung, confirmed by culture only
Conditions listed in A15.0, confirmed by culture only

A15.10 Tuberculosis of lung, with previously treated confirmation culture only (IM)

A15.11 Tuberculosis of lung, confirmed by biomolecular examination (IM)

A15.12 Tuberculosis of lung, with previously treated confirmed by biomolecular examination (IM)

Analisa Soal ditemukan

Riwayat Pengobatan 2021 - 2024

TCM sputum 10/10/2022 : MTb positif (+)

Rifampisin Resistan

TCM XDR 2/6/25 : MTb detected,

susceptible terhadap H, floroquinolon,
Amikasin, kanamisin, capromisin, etionami

Pemeriksaan TB dengan metode tes cepat berbasis biomolekuler (Tes Cepat Molekuler / TCM TB) sudah diperluas

KESIMPULAN : A15.12 (IM)

Tuberculosis of lung, with previously treated confirmed by biomolecular examination

Referensi :

https://www.tbindonesia.or.id/wp-content/uploads/2020/05/LAB_PETUNJUK-TEKNIS-PEMERIKSAAN-TB-DENGAN-TCM-2017.pdf



Xtreme Drug Resistance (XDR) / Multiple Drugs Resistance (U88)

Resistance, resistant (to)

- activated protein C [factor V Leiden mutation] D68.5
- antibiotic(s), by bacterial agent
 - - methicillin U80.1
 - - multiple U88
 - - penicillin U80.0

ICD 10 Revisi Tahun 2010

U88

Agent resistant to multiple antibiotics

Note: This category is provided for use when a bacterial agent is resistant to two or more antibiotics but there is insufficient detail to determine which antibiotic is contributing most to the "main condition". It should also be used for primary tabulation purposes when it is more convenient to record a single code; otherwise each specific antibiotic-resistant agent should be coded separately.

Catatan Draft ICD 10-IM

Resistance to antimicrobial and antineoplastic drugs (U82-U85) (IM)

Catatan :

Kategori ini tidak boleh digunakan dalam pengkodean utama. Kode disediakan untuk digunakan sebagai kode tambahan atau tambahan bila diinginkan untuk mengidentifikasi resistensi, non-responsif dan sifat bias suatu kondisi terhadap obat antimikroba dan antineoplastik.





Draft ICS Versi 1

Kode U88 masih ditambahkan untuk mencatat adanya resistensi obat

Jika terdapat resistensi terhadap Obat Anti-Tuberkulosis (OAT), kode primer tetap menggunakan A15–A19, dan ditambahkan:

- a. **Kode U88** untuk mencatat adanya resistensi obat, serta
- b. Kode tambahan **IM** yang menjelaskan jenis resistensi obat tertentu.

Namun demikian, kode A15–A19 tidak boleh digunakan apabila pasien dirawat karena mengalami efek samping akibat penggunaan OAT.





Kenapa Ditambahkan Kode IM U83.3 dan U84.33 ?

Sebagai Informasi Tambahan yang lebih spesifik terkait informasi resistensi obat antimikroba

DU : Tuberkulosis Paru Xtreme-Drug-Resistance (XDR)

Resistance : Rifampicin (R), Levofloxacin (Lfx), Moxifloxacin (Mfx),
Bedaquiline (Bdq), Clofazimine (Cfz)

Drug Types

U82 Resistance to betalactam antibiotics ^(IM)

Use additional code (B95-B98), if desired, to identify agents resistant to betalactam antibiotic treatment. ^(IM)

U82.20 Respiratory diseases with resistance of betalactamase (ESBL) resistance ^(IM)

U82.80 Respiratory diseases with resistance of betalactam antibiotics ^(IM)

U83 Resistance to other antibiotics ^(IM)

Use additional code (B95-B98), if desired, to identify agents resistant to other antibiotic treatment ^(IM)

U83.3 Respiratory diseases with resistance to other antibiotics ^(IM)

Episode of Care

U84 Resistance to other antimicrobial drugs ^(IM)

Use additional code (B95-B98), if desired, to identify agents resistant to other antibiotic treatment. ^(IM)

Excl.:

Resistance to antibiotics (U82-U83) ^(IM)

U84.10 Respiratory diseases with resistance to antifungal drugs ^(IM)

U84.20 Respiratory diseases with resistance to antiviral drugs ^(IM)

U84.31 Tuberculosis with multidrug resistance ^(IM)

U84.32 Tuberculosis with pre extreme drug resistance ^(IM)

U84.33 Tuberculosis with extreme drug resistance ^(IM)

U84.34 Tuberculosis with poly drug resistance ^(IM)

U84.35 Tuberculosis with mono drug resistance ^(IM)





Kenapa **Bronchiectasis (J47)** tidak dikodekan?

VOLUME III

Bronchiectasis (cylindrical) (diffuse)
(fusiform) (localized) (saccular) **J47**

- congenital **Q33.4**
- tuberculous NEC (*see also*
Tuberculosis, pulmonary) **A16.2**

VOLUME I

A15.0 **Tuberculosis of lung, confirmed by sputum microscopy
with or without culture**

Tuberculous:

- **bronchiectasis**
- fibrosis of lung
- pneumonia
- **pneumothorax**

} confirmed by sputum microscopy with
or without culture





JAWABAN SOAL 8

	Nama Diagnosis	Kode ICD-10
Diagnosis Utama	Follow Up Gigi impaksi mandibula kanan bawah dengan perikoronitis kronis ringan (Gigi 48)	Z09.8 K01.17 (IM) K05.30 (IM) KG48 (IM)
Diagnosis Sekunder	1. DM tipe 2 2. Hipertensi Terkontrol	E11.9 I10





Gigi impaksi mandibula kanan bawah (K01.17) (IM)

impaction, impacted

- bowel, colon or rectum (fecal) [K56.4](#)
- calculus — see [Calculus](#)
- cerumen (ear) (external) [H61.2](#)
- dental [K01.1](#)
- fecal, feces [K56.4](#)
- fracture — see [Fracture, by site](#)
- gallbladder (see also [Cholelithiasis](#)) [K80.2](#)
- gallstone(s) (see also [Cholelithiasis](#)) [K80.2](#)
- - bile duct (common) (hepatic) [K80.5](#)
- - cystic duct [K80.2](#)
- - in intestine, with obstruction (any part) [K56.3](#)
- intestine (calculous) (fecal) NEC [K56.4](#)
- - gallstone, with ileus [K56.3](#)
- molar — see [Impaction, tooth](#)
- shoulder [O66.0](#)
- - affecting fetus or newborn [P03.1](#)
- - causing obstructed labor [O66.0](#)
- **tooth, teeth [K01.1](#)**
- - with abnormal position (same or adjacent tooth) [K07.3](#)
- turbinate [J34.8](#)

K01 Embedded and impacted teeth

Excludes: embedded and impacted teeth with abnormal position of such teeth or adjacent teeth ([K07.3](#))

K01.0 Embedded teeth

An embedded tooth is a tooth that has failed to erupt without obstruction by another tooth.

K01.1 Impacted teeth

An impacted tooth is a tooth that has failed to erupt because of obstruction by another tooth.

Volume I
ICD-10
Th. 2010

Pericoronitis kronis ringan K05.30 (IM)

- trachea [J39.8](#)

Pericoronitis (chronic) [K05.3](#)
- acute [K05.2](#)

Peridiverticulitis (intestine) (see also [Diverticula](#)) [K57.9](#)

Periendocarditis (see also [Endocarditis](#))
[I38](#)

Periepididymitis (see also [Epididymitis](#))
[N45.9](#)

Perifolliculitis [L08.8](#)
- capitis abscedens (et suffodiens) [L66.3](#)
- superficial pustular [I01.0](#)

K05.3

Chronic periodontitis

Chronic pericoronitis

Periodontitis:

- NOS
- complex
- simplex

K05.3 Chronic periodontitis
Chronic pericoronitis
Periodontitis:

- NOS
- complex
- simplex

K05.30 Chronic periodontitis, Simplex (IM)

K05.31 Chronic periodontitis, Complex (IM)

K05.32 Chronic periodontitis, Chronic pericoronitis (IM)

K05.33 Chronic periodontitis, Thickened follicle (IM)

K05.38 Other specified chronic periodontitis (IM)

K05.39 Chronic periodontitis, unspecified (IM)

Kode ICD-10-IM untuk IMPAKSI GIGI

- K01.10** Impacted teeth, Maxillary incisor (IM)
- K01.11** Impacted teeth, Mandibular incisor (IM)
- K01.12** Impacted teeth, Maxillary canine (IM)
- K01.13** Impacted teeth, Mandibular canine (IM)
- K01.14** Impacted teeth, Maxillary premolar (IM)
- K01.15** Impacted teeth, Mandibular premolar (IM)
- K01.16** Impacted teeth, Maxillary molar (IM)
- K01.17** Impacted teeth, Mandibular molar (IM)
- K01.18** Impacted teeth, Supernumerary tooth (IM)
- K01.19** Impacted tooth, unspecified (IM)

Lateralisasi gigi 48 **KG48**

K01 Embedded and impacted teeth

Draft ICD-10-IM

"Use additional code for the lateral side of the tooth"

Excl.:

embedded and impacted teeth with abnormal position of such teeth or adjacent teeth ([K07.3](#))

K01.0 Embedded teeth

An embedded tooth is a tooth that has failed to erupt without obstruction by another tooth.

K01.1 Impacted teeth

An impacted tooth is a tooth that has failed to erupt because of obstruction by another tooth.

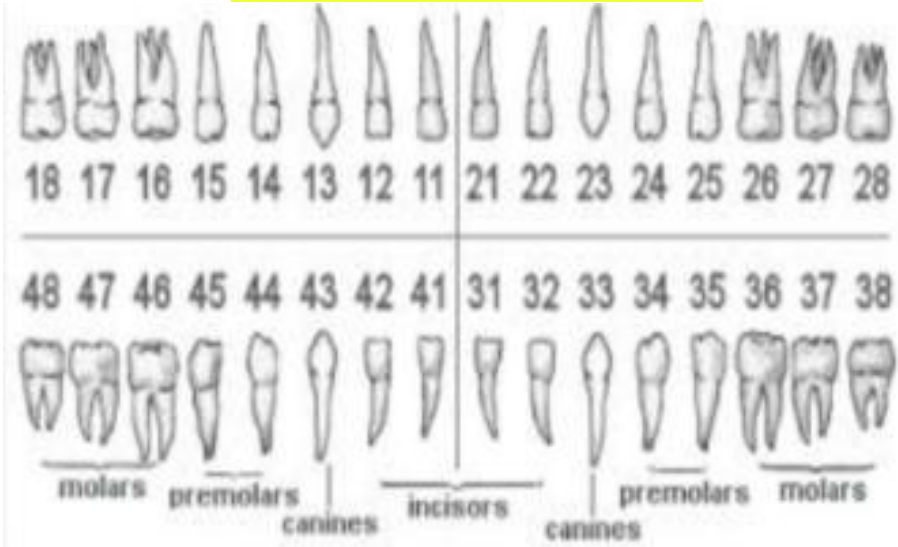
For Permanent Teeth / Adult Teeth :

- | | | | |
|------|----------------------------------|-------------|-------------------------------------|
| KG11 | Upper Right Central Incisor (IM) | KG31 | Lower left central incisor (IM) |
| KG12 | Upper right lateral incisor (IM) | KG32 | Lower left lateral incisor (IM) |
| KG13 | Upper right canine (IM) | KG33 | Lower left canine (IM) |
| KG14 | Upper right first premolar (IM) | KG34 | Lower left first premolar (IM) |
| KG15 | Upper right second premolar (IM) | KG35 | Lower left second premolar (IM) |
| KG16 | Upper right first molar (IM) | KG36 | Lower left first molar (IM) |
| KG17 | Upper right second molar (IM) | KG37 | Lower left second molar (IM) |
| KG18 | Upper right third molar (IM) | KG38 | Lower left third molar (IM) |
| KG21 | Upper left central incisor (IM) | KG41 | Lower right central incisor (IM) |
| KG22 | Upper left lateral incisor (IM) | KG42 | Lower right lateral incisor (IM) |
| KG23 | Upper left canine (IM) | KG43 | Lower right canine (IM) |
| KG24 | Upper left first premolar (IM) | KG44 | Lower right first premolar (IM) |
| KG25 | Upper left second premolar (IM) | KG45 | Lower right second premolar (IM) |
| KG26 | Upper left first molar (IM) | KG46 | Lower right first molar (IM) |
| KG27 | Upper left second molar (IM) | KG47 | Lower right second molar (IM) |
| KG28 | Upper left third molar (IM) | KG48 | Lower right third molar (IM) |



NOMENKLATUR GIGI

MAXILLA / UPPER JAW



MANDIBLE / LOWER JAW

KODE LATERALISASI GIGI

For Permanent Teeth / Adult Teeth :

- KG11 Upper Right Central Incisor (IM)
- KG12 Upper right lateral incisor (IM)
- KG13 Upper right canine (IM)
- KG14 Upper right first premolar (IM)
- KG15 Upper right second premolar (IM)
- KG16 Upper right first molar (IM)
- KG17 Upper right second molar (IM)
- KG18 Upper right third molar (IM)
- KG21 Upper left central incisor (IM)
- KG22 Upper left lateral incisor (IM)
- KG23 Upper left canine (IM)
- KG24 Upper left first premolar (IM)
- KG25 Upper left second premolar (IM)
- KG26 Upper left first molar (IM)
- KG27 Upper left second molar (IM)
- KG28 Upper left third molar (IM)

- KG31 Lower left central incisor (IM)
- KG32 Lower left lateral incisor (IM)
- KG33 Lower left canine (IM)
- KG34 Lower left first premolar (IM)
- KG35 Lower left second premolar (IM)
- KG36 Lower left first molar (IM)
- KG37 Lower left second molar (IM)
- KG38 Lower left third molar (IM)
- KG41 Lower right central incisor (IM)
- KG42 Lower right lateral incisor (IM)
- KG43 Lower right canine (IM)
- KG44 Lower right first premolar (IM)
- KG45 Lower right second premolar (IM)
- KG46 Lower right first molar (IM)
- KG47 Lower right second molar (IM)
- KG48 Lower right third molar (IM)





1. Diabetes Mellitus Tipe 2 **E11.9**

Diabetes, diabetic (mellitus) (controlled)
(familial) (severe) **E14.-**

- acetonemia — *code to E10-E14 with fourth character .1*
- acidosis — *code to E10-E14 with fourth character .1*
- adult-onset (nonobese) (obese) **E11.-**
- type I **E10.-**
- type II (nonobese) (obese) **E11.-**
- unstable **E10.-**



E11

Non-insulin-dependent diabetes mellitus

[See before E10 for subdivisions.]

Includes: diabetes (mellitus)(nonobese)(obese):

- adult-onset
- maturity-onset
- nonketotic
- stable
- type II

- .7 With multiple complications**
- .8 With unspecified complications**
- .9 Without complications**





JAWABAN SOAL 9

	Nama Diagnosis	Kode ICD-10
Diagnosis Utama	Katarak traumatik OD	H26.1
Diagnosis Sekunder	- DM tipe 2 - Hipertensi - Pasca Terkena Dahan Pohon yg jatuh Ketika Bersih-Bersih Desa 1 bulan yll (External Causes) <i>1. Kode Lateralitas Mata Kanan OD = Optic Dextra</i>	E11.9 I10 W20.03 - - HL00 (IM)





Katarak Traumatik **H26.1**

Cataract-----*continued*

- - punctate **H25.0**
- - specified NEC **H25.8**
- - subcapsular polar (anterior)
(posterior) **H25.0**
- toxic **H26.3**
- traumatic **H26.1**

H26.1 **Traumatic cataract**

Use additional external **cause code**
(Chapter XX), if desired, to
identify cause

Chapter XX

**External causes of morbidity and
mortality
(V01-Y98)**





Pasca Terkena Dahan Pohon yang Jatuh

Ketika Bersih-Bersih Desa di Perumahan W20.03

Section II External Causes Of Injury

- ▶ Section I Alphabetical Index Of Diseases And Nature Of Injury
- ▶ **Section II External Causes Of Injury**
- ▶ Section III Table Of Drugs And Chemicals

Fall, falling.....Continued

- object (see also *Struck by, object, falling*) W20.-

Struck (accidentally) by...Continued

- object NEC W22.-
- - falling W20.-
- - - from, in, on

Exposure to inanimate mechanical forces (W20-W49)

[\[See at the beginning of this chapter for fourth-character subdivisions\]](#)

W20 **Struck** by thrown, projected or falling object

Includes: cave-in without asphyxiation or suffocation
collapse of building, except on fire

falling:

- rock
- stone
- tree

Kode Lokasi

.0 Home

Apartment
Boarding-house
Caravan [trailer] park, residential
Farmhouse
Home premises
House (residential)

Kode Aktivitas

3 While engaged in other types of work

Domestic duties such as:

- caring for children and relatives
- **cleaning**
- cooking
- gardening
- household maintenance

Duties for which one would not normally gain an income
Learning activities, e.g. attending school session or lesson
Undergoing education

Diabetes Mellitus Tipe 2 **E11.9**

Diabetes, diabetic (mellitus) (controlled)
(familial) (severe) **E14.-**

- acetonemia — code to **E10-E14** with fourth character .1
- acidosis — code to **E10-E14** with fourth character .1
- adult-onset (nonobese) (obese) **E11.-**
- arising in pregnancy **O24.4**
- type I **E10.-**
- type II (nonobese) (obese) **E11.-**
- unstable **E10.-**



E11 **Non-insulin-dependent diabetes mellitus**

[See before **E10** for subdivisions]

Includes: diabetes (mellitus)(nonobese)(obese):

- adult-onset
- maturity-onset
- nonketotic
- stable
- type II

· neuropathic (**E11.4.0**)

.7 With multiple complications

.8 With unspecified complications

.9 Without complications



Hipertensi terkontrol (**I10**)

Hypertension, hypertensive

(accelerated) (benign) (essential)
(idiopathic) (malignant) (primary)
(systemic) **I10**

- with
- - heart involvement (conditions in **I51.4-I51.9** due to hypertension)
(see also *Hypertension, heart*) **I11.9**
- - kidney involvement (see also **I12.0**)

I10 **Essential (primary) hypertension**

High blood pressure

Hypertension

(arterial)(benign)(essential)(malignant)(primary)(systemic)

Excludes: involving vessels of:

- brain (**I60-I69**)
- eye (**H35.0**)



KODE LATERALITAS MATA

Optic Dextra (OD) = Mata Kanan = HL00

List Code ICD 10 2010 Indonesian Modification (IM)

Daftar Kode Diagnosa Tambahan Dalam Indonesian Modification (IM)

Chapter VII

Diseases of the eye and adnexa (H00-H59)

DRAFT ICD-9-IM

Use Additional Code to Identify Eye Anatomy

Note : Digunakan Untuk menentukan lateralisasi mata agar dokumentasi medis tergambar dalam kodifikasi diagnosa penyakit (jika berada di mata kanan atau kiri maka gunakan kodenya mata kanan dan kiri secara bersamaan)

HL00 Right Eye ^(IM)

HL01 Left Eye ^(IM)





JAWABAN SOAL 10

	Nama Diagnosis	Kode ICD-10
Diagnosis Utama	Cerebral Venous Sinus Thrombosis (CVST)	I67.6
Diagnosis Sekunder	-	-





Cerebral Venous Sinus Thrombosis (CVST) **I67.6**

Thrombosis, thrombotic (multiple)
(progressive) (septic) (vein) (vessel)

I82.9

- brain (artery) (stem) (see also *Occlusion, artery, cerebral*) **I66.9**
- - due to syphilis **A52.0† I68.8***
- - puerperal, postpartum, childbirth **O99.4**
- - sinus (see also *Thrombosis, intracranial venous sinus*) **G08**
- intracranial (arterial) **I66.9**
- - venous sinus (any) **G08**
- - - nonpyogenic origin **I67.6**

I67.6



Nonpyogenic thrombosis of intracranial venous system

Nonpyogenic thrombosis of:

- cerebral vein
- intracranial venous sinus

Excludes: when causing infarction (**I63.6**)





Kenapa CVST Non-Pyogenic (I67.6) ?

Kenapa Tidak CVST Infark (I63.6) ?

Kenapa Tidak CVST Pyogenic (G08) ?



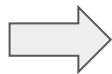
EXPERTISE RADIOLOGI

Nama Pemeriksaan

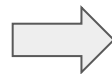
MRV (Magnetic Resonance Venography) Cerebral

Hasil / Temuan

- Tampak hilangnya aliran (flow void) pada sinus sagittalis superior, sinus transversus kanan, dan sebagian sinus sigmoid kanan, sesuai gambaran trombosis vena serebral tidak terlihat adanya pus/nanah. - Tampak peningkatan sinyal T1/T2 pada sinus tersebut. - Sinus transversus kiri dan vena internal serebral kiri masih tampak normal. - Parenkim otak dalam batas normal, tidak tampak edema difus maupun infark vena yang nyata. - Ventrikel lateral simetris, garis tengah tidak bergeser.



G08 kurang tepat



I63.6 kurang tepat

Kesan MRV

Gambaran sesuai dengan Cerebral Venous Sinus Thrombosis (CVST) melibatkan: • Sinus sagittalis superior • Sinus transversus kanan • Sinus sigmoid kanan, tanpa pus atau nanah.

KESIMPULAN : CVST Non-Pyogenic (I67.6)





TERIMA KASIH

